Perioperative pain research portfolio index

Independent educational portfolio by Collin Blaine George. Updated 27 October 2026.

The projects below are literature analyses, study exercises and a software simulation. They do not establish clinical competency, institutional endorsement or a validated patient-care tool.

Project: Non-opioid anesthesia overview
Current work: Selected-source educational overview; unsupported historical review-method claims withdrawn
Access: Private; publication review pending

Project: TKA evidence map
Current work: Selected-source narrative map; estimated screening counts, unverified extraction tables and original pooling withdrawn
Access: Public educational draft

Project: Breast reconstruction analgesia review
Current work: Narrative analysis with reference-identity records, search diagnostics and explicit population limits
Access: Public educational draft

Project: Breast perioperative learning modules
Current work: Discussion modules that replace the earlier operational protocol
Access: Private; publication review pending

Project: Critical care physiology
Current work: 42 entirely synthetic reasoning exercises; no real-patient encounters
Access: Private; publication review pending

Project: Smart NOA
Current work: Unvalidated rule-checking simulation with no device or patient interface
Access: Private; publication review pending

Private links require repository access. Their listing here does not make the source public.

Research interests

Perioperative analgesia, pain mechanisms, longer-term recovery and persistent postsurgical pain are areas for further study. Proposed studies are plans; no participant recruitment, institutional approval, completed dissertation or clinical activity is represented here.

Review and provenance

Earlier unsupported clinical, methodological and institutional claims are superseded by the current source. Publication review and provenance describe the completed checks and remaining limits. The corrected source does not certify legal compliance or authorize patient care.

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